Lorlatinib therapy

Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous lorlatinib · individual supportive care

Active medicines: lorlatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP rates this oral regimen minimal to low emetogenic risk; follow local oral-systemic anticancer therapy (SACT) prevention/rescue policy. NCIS V8 describes parenteral antiemetics and is not copied as a daily oral schedule.
- No routine premedications are required. Lipid-lowering, blood pressure (BP)/glucose and symptom treatment are individual prescriptions reviewed for interactions.
- If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.
- Lorlatinib may harm an unborn baby and make hormonal contraception less effective. Women who could become pregnant need highly effective non-hormonal contraception during treatment and for at least 35 days afterwards; discuss any unavoidable hormonal method and barrier protection. Men with partners who could become pregnant need effective contraception including a condom during treatment and for at least 14 weeks afterwards; condoms are also needed with a pregnant partner. Discuss fertility preservation before treatment. Do not breastfeed during treatment; agree when it can safely resume with the team.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antimicrobial prophylaxis or steroid course is specified in NCCP 00570.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00570 v3 · p5–6 · NCCP 00570 v3 supportive care; NCIS V8 scope checked

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required